

Patient ID: EGPatient ID: EGPeriod: March 26 to April 25, 2024Report Date: April 26, 2024Coverage: Bed: 552/744h (74.2%)Chair: 395/744h (53.1%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing		High Breathing	
5 PM to 6 PM		4 PM to 5 PM	
Vital Stat	Avg	Min	Max
Heart Rate	60 bpm	49 bpm	74 bpm
Breathing	18 brpm	11 brpm	46 brpm

Last 24 hours: one high breathing episode in the afternoon; one elevated breathing episode in the evening.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
5 PM to 6 PM	2h	2	Elevated Breathing	29	29	30	Check SpO2 and lung sounds; assess for fluid overload or early infection
4 PM to 5 PM	1h	1	High Breathing (brief)	38	31	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

Patient clinical status over 31 days from Mar 26 to Apr 25

HRBreathing

Elevated Breathing Mar 27 to Apr 04 (9d)	Elevated Breathing Apr 11 to Apr 17 (7d)	Elevated Breathing Apr 20 to Apr 25 (6d)	High Breathing Apr 11 to Apr 11 (1d)	Low HR Mar 29 to Mar 29 (1d)	Very High Breathing Apr 16 to Apr 16 (1d)
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29 episodic events Detected, spanning 35h total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate, Very High Breathing.
Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
29	35h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 16	11 AM to 12 PM	1h	1	Very High Breathing	46	46	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Apr 11	10 AM to 11 AM	2h	2	High Breathing	38	22	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 29	4 AM to 6 AM	3h	2	Low Heart Rate	44	41	46	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 27	12 PM to 2 PM	11h	8	Elevated Breathing	27	19	40	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 11	2 PM to 3 PM	11h	9	Elevated Breathing	26	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 20	6 PM to 7 PM	7h	7	Elevated Breathing	28	16	44	Check SpO2 and lung sounds; assess for fluid overload or early infection

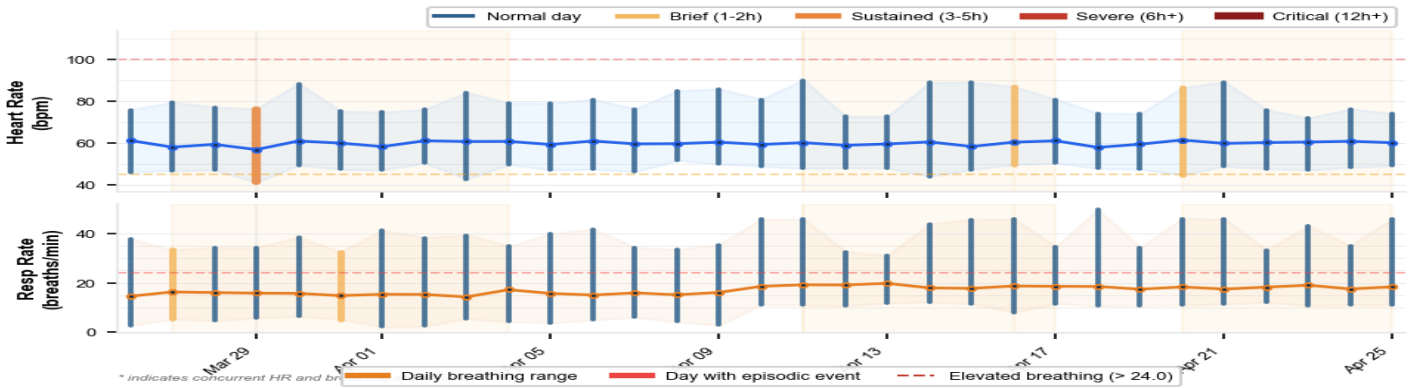
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 40 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- High Breathing (avg 38 brpm, peak 46 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 46 brpm, peak 46 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 16 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

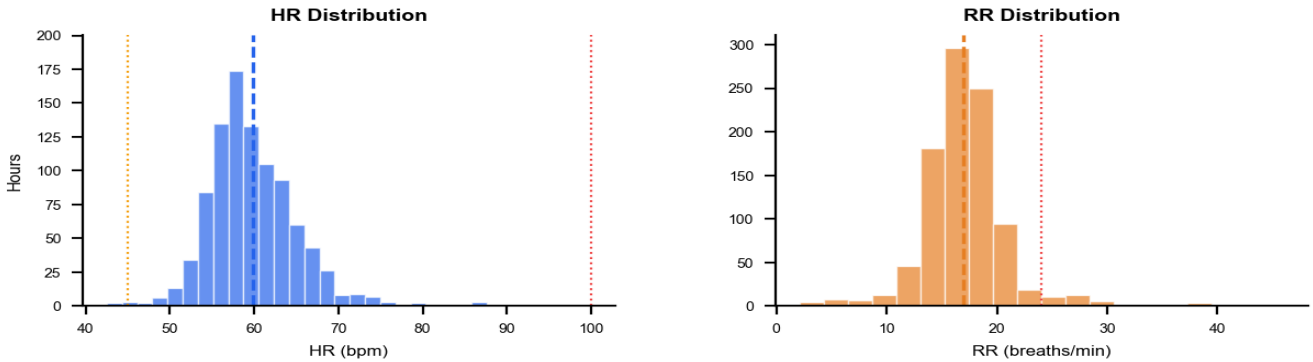
Clinical Pattern Observations:

- 1. **Breathing variability:** RR spread 10 brpm (11 to 22).
- 2. **Clustered pattern:** Episodic events on 5 of 31 days (16%).
- 3. **Daytime pattern:** Episodes concentrated during waking hours.

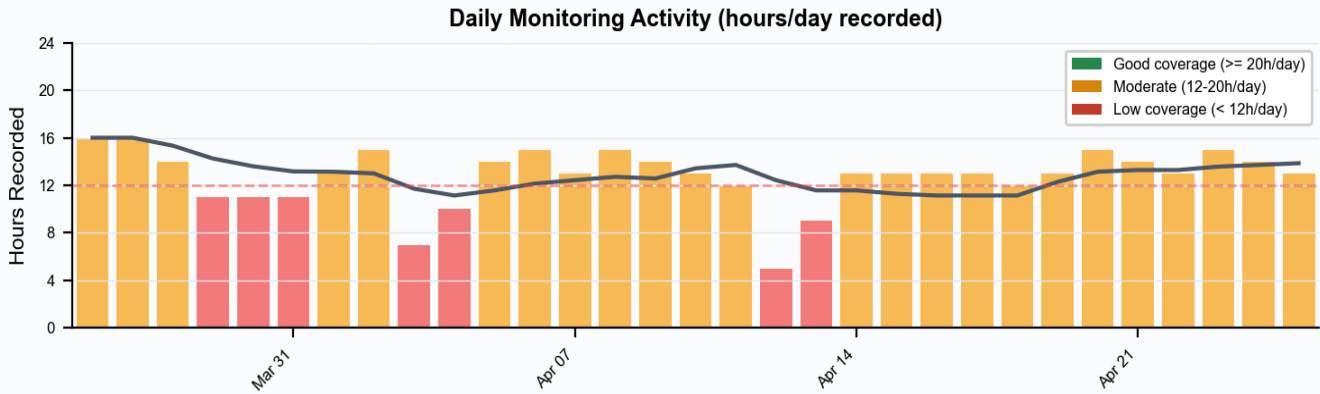


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.